AI-Assisted Clinical Documentation Review Package

Training encounter: Eleanor Vance | Encounter date: October 14, 2026 | Provider: Marcus Thorne, MD | Encounter ID: ENC-2026-1014-884 | EMR: PracticeOne v12.4

Review basis: Source Conversation Transcript; unreviewed Dragon Copilot Draft Clinical Encounter Note; EMR Post-Transfer Chart Scenario. This is a simulated quality-assurance work sample; proposed corrections require clinician verification before finalization.

Section 1 - Annotated Clinical Encounter Note

The text below preserves the original clinical sections and supplies explicit, source-grounded replacement text. Each annotation identifies the documentation standard addressed.

History / Subjective

Chief complaint (retain): Follow-up for type 2 diabetes and essential hypertension.

Replace HPI with: 68-year-old woman returning for diabetes and blood-pressure follow-up approximately four weeks after lisinopril was increased. Reports new, brief morning dizziness on standing quickly, resolving within seconds. Today's blood pressure is 118/74 mmHg; home blood pressures average approximately 114/70 mmHg, compared with 142/88 mmHg before the lisinopril increase. Plans a two-week Alaska cruise next month and asks about medication precautions while traveling.

Annotations H1-H3: Add the four-week medication-titration context and positional symptom duration (**completeness**); add comparative home and pretitration readings to explain the medication decision (**clinical specificity**); frame cruise discussion as a request for travel medication advice rather than an undocumented formal clearance determination (**clinical encounter relevance**).

Objective

Retain: BP 118/74 mmHg; HbA1c 6.8%, down from 7.4%; fasting glucose log approximately 115 mg/dL. **Add from transcript:** Home BP average approximately 114/70 mmHg; earlier BP 142/88 mmHg before dose increase.

Annotation O1: Include the comparison values that the clinician used in making the antihypertensive adjustment (**clinical specificity**).

Assessment

Replace hypertension assessment with: Blood pressure has decreased after the recent lisinopril increase, with new brief dizziness on standing. Clinician discussed the lower readings and planned to reduce lisinopril to 10 mg daily.

Replace diabetes assessment with: Type 2 diabetes with improved glycemic readings (HbA1c 6.8% from 7.4%; fasting glucose approximately 115 mg/dL), doing well on metformin ER without reported stomach upset.

Annotations A1-A2: Tie the dizziness and treatment change to the documented comparison readings rather than asserting a separately measured orthostatic blood-pressure drop (**clinical specificity**); add reported medication tolerability supporting continuation (**completeness**).

Plan

1. Hypertension: ~~Decrease lisinopril from 20 mg to 5 mg daily~~ **Correct to: Reduce lisinopril from 20 mg to 10 mg orally once daily, starting tomorrow morning.** Clinician linked the change to lower blood-pressure readings and standing-related dizziness.

2. Diabetes: Continue metformin ER 1000 mg twice daily without change.

3. Follow-up: ~~Return in 6 months~~ **Correct to: Arrange follow-up in 8 weeks** to reassess blood pressure on lisinopril 10 mg daily and review a basic metabolic panel. If dizziness fails to improve following dose reduction, call the clinic right away.

4. Travel advice: Pack medications in carry-on luggage, bring several extra days' supply, and keep glucose-testing supplies accessible. Clinician stated the patient was fine to travel.

Annotations P1-P4: Correct lisinopril dose (**clinical specificity**); replace the wrong follow-up interval and preserve the linked BP/BMP review (**clinical specificity and completeness**); retain the clinician's immediate call instruction (**clinical encounter relevance**); include the full, explicitly discussed travel advice (**completeness**). No independent clinical orders are authorized by these proposed

edits.

Section 2 - Discrepancy Log

The source transcript captures the items below. Therefore these are **draft errors**, not proven ambient capture losses. The provided materials do not document a manual correction that introduced an error; no edit error is asserted.

Entry 1

Chain Location (Error Type): Draft error - source transcript to AI Plan

Specific Content Affected: Lisinopril dose: transcript says reduce 20 mg to **10 mg daily** starting tomorrow; draft says **5 mg daily**.

Clinical or Record Impact: The draft could prompt an incorrect medication instruction; EMR free text repeats the 5 mg error, increasing reconciliation risk.

Entry 2

Chain Location (Error Type): Draft error - source transcript to AI Plan

Specific Content Affected: Return interval: clinician specified **8 weeks** for BP reassessment and basic metabolic panel; draft specifies **6 months**.

Clinical or Record Impact: Could delay reassessment after a medication adjustment; the EMR structured follow-up field also contains 6 months.

Entry 3

Chain Location (Error Type): Draft error - source transcript to AI History / Objective

Specific Content Affected: Missing context: four-week interval since lisinopril increase, home BP approximately **114/70**, and earlier BP **142/88**.

Clinical or Record Impact: The longitudinal BP evidence and reason for dose adjustment become less clear to subsequent reviewers.

Entry 4

Chain Location (Error Type): Draft error - source transcript to AI Plan / History

Specific Content Affected: Travel counseling omitted from plan: carry-on medications, extra days' supply, and accessible testing supplies; only a general trip request appears in history.

Clinical or Record Impact: The note does not completely preserve the advice given; transferred AVS contains carry-on advice but omits the extra supply and accessible testing supplies.

Entry 5

Chain Location (Error Type): Draft error - source transcript to AI Assessment

Specific Content Affected: Metformin tolerability detail omitted: patient had not reported stomach upset while taking metformin ER 1000 mg twice daily.

Clinical or Record Impact: Removes a relevant piece of evidence supporting the documented decision to continue therapy unchanged.

Entry 6

Chain Location (Error Type): Draft error - source transcript to AI Plan

Specific Content Affected: Callback instruction weakened: draft says call if dizziness worsens or fails to resolve; transcript specifically says call **right away if it does not improve after lowering lisinopril**.

Clinical or Record Impact: Timing and condition for follow-up contact are less specific than the clinician's actual instruction.

Section 3 - EMR Record Integrity Evaluation

Field appropriateness - finding: The post-transfer chart contains a lisinopril reduction to 5 mg in the free-text Clinical Notes field, but its separate Medication Change/Orders field is blank and the active medication table still lists lisinopril 20 mg as active. The chart also includes a six-month value in its structured follow-up field, derived from the erroneous draft. **Implication:** The note text, medication order area, active medication table, and scheduled follow-up representation do not express a consistent clinician-directed plan. A free-text statement is not a substitute for correctly entered and clinician-authorized discrete medication orders. The six-month structured interval also needs correction to match the eight-week discussion.

Traceability - finding: The transcript documents a reduction from 20 mg to 10 mg starting the next morning and review at eight weeks, whereas the Dragon Copilot draft substitutes 5 mg and six months; those incorrect draft values then appear in the EMR free text and structured follow-up field. The chart attributes primary and secondary diagnosis codes to AI draft auto-population, but the scenario does not provide a clinician attestation or independent verification of the final discrete mapping. **Implication:** Although the source of the discrepancies can be reconstructed from the three supplied documents, the chart as shown is not an accurate, complete audit-ready record of the clinician's decisions. Source-to-draft and draft-to-chart reconciliation must be documented before signing.

Downstream usability - finding: A future clinician reviewing only the active medication list could see lisinopril 20 mg; one reading the pasted note could see 5 mg; the actual spoken decision was 10 mg. Similarly, scheduling staff could rely on the six-month structured interval rather than the eight-week follow-up. The after-visit summary mentions carry-on medication packing and six-month follow-up but omits the extra supply and accessible testing supplies advised in the transcript. **Implication:** Medication reconciliation, scheduling, patient instructions, and continuity of care could be affected even when different staff members reasonably use their respective EMR fields. The downstream-facing fields must be brought into agreement with the clinician-confirmed encounter before release.

Section 4 - Final Disposition Recommendation

Disposition: ESCALATE for clinician-directed correction and hold sign-off. This is a pre-signature chart (status: pending final clinician reconciliation and sign-off). The coordinator should not independently authorize the drug dose, modify signed clinical judgments, or mark the record final.

The escalation is supported first by **Discrepancy Log Entry 1 (lisinopril 5 mg versus 10 mg)** and **Entry 2 (six months versus eight weeks)**, two clinically significant deviations from the recorded conversation. It is also supported by the **field appropriateness finding**: the Medication Change/Orders field is blank and the active lisinopril entry remains 20 mg, despite the change in the encounter. The **downstream usability finding** shows that different EMR views would give staff and the patient conflicting directions.

Before sign-off, route the annotated note and discrepancy log to Dr. Thorne for confirmation; reconcile the note wording, authorized medication order and active medication list, eight-week follow-up, and after-visit instructions; then perform a second source-to-draft-to-EMR check and record reviewer approval. Only the responsible clinician should confirm the final clinical content and orders. Do not transmit or rely on the inconsistent AVS as final patient guidance. Once these corrections are verified, the encounter may return to the normal sign-off workflow.

Section 3 + Section 4 combined narrative word count: approximately 527 words.

Evidence references: Source Conversation Transcript (October 14, 2026), especially clinician statements regarding BP, medication change, diabetes, travel advice, and eight-week follow-up; Dragon Copilot Draft Clinical Encounter Note; EMR Post-Transfer Chart Scenario (PracticeOne v12.4).